

Breaking prolonged sitting reduces postprandial glycemia in healthy, normal-weight adults: a randomized crossover trial.

BACKGROUND: Sedentary behavior is a risk factor for cardiometabolic disease. Regularly interrupting sedentary behavior with activity breaks may lower this risk. **OBJECTIVE:** We compared the effects of prolonged sitting, continuous physical activity combined with prolonged sitting, and regular activity breaks on postprandial metabolism. **DESIGN:** Seventy adults participated in a randomized crossover study. The prolonged sitting intervention involved sitting for 9 h, the physical activity intervention involved walking for 30 min and then sitting, and the regular-activity-break intervention involved walking for 1 min 40 s every 30 min. Participants consumed a meal-replacement beverage at 60, 240, and 420 min. **RESULTS:** The plasma incremental area under the curve (iAUC) for insulin differed between interventions (overall $P < 0.001$). Regular activity breaks lowered values by $866.7 \text{ IU} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ (95% CI: 506.0, 1227.5 $\text{IU} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$; $P < 0.001$) when compared with prolonged sitting and by $542.0 \text{ IU} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ (95% CI: 179.9, 904.2 $\text{IU} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$; $P = 0.003$) when compared with physical activity. Plasma glucose iAUC also differed between interventions (overall $P < 0.001$). Regular activity breaks lowered values by $18.9 \text{ mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ (95% CI: 10.0, 28.0 $\text{mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$; $P < 0.001$) when compared with prolonged sitting and by $17.4 \text{ mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ (95% CI: 8.4, 26.3 $\text{mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$; $P < 0.001$) when compared with physical activity. Plasma triglyceride iAUC differed between interventions (overall $P = 0.023$). Physical activity lowered values by $6.3 \text{ mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ (95% CI: 1.8, 10.7 $\text{mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$; $P = 0.006$) when compared with regular activity breaks. **CONCLUSION:** Regular activity breaks were more effective than continuous physical activity at decreasing postprandial glycemia and insulinemia in healthy, normal-weight adults. This trial was registered with the Australian New Zealand Clinical Trials registry as ACTRN12610000953033.